



Pediatric and Adult Influenza

Clinical Case Quiz

An 18-year-old, otherwise healthy male presents with 24 hours of fever 103F, body aches, chills, sore throat, cough, congestion, and fatigue. There are no red flag symptoms otherwise.

He was exposed to a documented case of influenza in the home and is asking whether oseltamivir (Tamiflu) is recommended.

What is the next best step?

- a. Recommend that he take a home influenza test and make another appointment to review the results.
- b. Direct him to an in-person appointment or nurse clinic for further evaluation and testing.
- c. Prescribe oseltamivir to pick up, only if symptoms have not improved after 3 days.
- d. Prescribe oseltamivir immediately after discussing risks and benefits.
- e. Recommend supportive care only, after discussing risks and benefits of oseltamivir.

Answer: Both d and e are appropriate



Discussion: Choosing the Best Path for Care

A. "Recommend that he take a home influenza test and make another appointment to review the results."

Teaching Points

- This is **NOT** required when influenza is circulating in the community and the patient has classic influenza-like symptoms. In this particular case, he also has known exposure to influenza.
- Delaying treatment to wait for test results may reduce the effectiveness of antiviral therapy, which is best initiated within 48 hours of symptom onset.
- Asking him to make a follow-up appointment also further utilizes healthcare resources unnecessarily.

B. "Direct him to an in-person appointment or nurse clinic for further evaluation and testing."

Teaching Points

- This patient has no red flag symptoms or signs of complications that would require an in-person appointment.



- Red flag symptoms would be shortness of breath, persistent chest pain, confusion, severe dehydration, or severe progressive illness requiring physical examination.

- Directing him to in-person care may also delay treatment, increase exposure risk to others, and utilize healthcare resources unnecessarily.

- Our local nurse clinics do **NOT** have a process for POCT influenza testing. Do **NOT** send patients to nurse clinics for POCT flu/ COVID testing. They will be turned away.

C. "Prescribe oseltamivir to pick up only if symptoms have not improved after 3 days."

Teaching Points

- This is **NOT** recommended. Oseltamivir is most effective when started within 48 hours of symptom onset.
- There is little to **NO** benefit when oseltamivir is started beyond this window in low-risk patients.

D. "Prescribe oseltamivir immediately after discussing risks and benefits."

Teaching Points: Oseltamivir Clinical Guidance

- **Optional Treatment:** Reasonable but not mandatory for this low-risk, healthy patient.
- **Timing:** Patient is within the 48-hour window of symptom onset with known exposure.
- **Shared Decision-Making:** Weigh modest clinical benefits against potential side effects with the patient.
- **Risk Profile:** Greatest efficacy is reserved for high-risk patients or those with significant comorbidities.



E. "Recommend supportive care only, after discussing risks and benefits of oseltamivir."

Teaching Points

- This is an equally appropriate option for low-risk patients.
- Oseltamivir has only modest benefits for low-risk patients (~1 day symptom reduction) and does **NOT** significantly reduce hospitalization risk in this population.
- Supportive care alone is safe and effective for low-risk patients, and patients should be counseled on return precautions.

High-Risk Conditions for Influenza Complications

High-Risk Conditions for Influenza Complications	
Adults	Children
Age ≥65 years	Per AAP, <5 years old (especially those <2 years)
Pregnancy or ≤2 weeks postpartum	Chronic lung disease (asthma, cystic fibrosis)
Chronic lung disease (asthma, COPD)	Heart disease (except hypertension alone)
Heart disease (except hypertension alone)	Diabetes mellitus
Diabetes mellitus	Immunosuppression (HIV, medications, cancer)
Immunosuppression (HIV, medications, cancer)	Chronic kidney or liver disease
Chronic kidney or liver disease	Neurologic/neurodevelopmental disorders
Neurologic/neurodevelopmental disorders	Obesity (≥95 th percentile)
Obesity (BMI ≥40)	Children on long-term aspirin therapy
Nursing home/long-term care patients	Conditions affecting respiratory function
American Indian/Alaska Native	American Indian/Alaska Native

References

Centers for Disease Control and Prevention. (2026). Influenza. *CDC Yellow Book*. <https://www.cdc.gov/yellow-book/index.html>

Committee on Infectious Diseases. (2025). Recommendations for prevention and control of influenza in children, 2025-2026: Technical report. *American Academy of Pediatrics*. <https://publications.aap.org/pediatrics/article/156/6/e2025073622/202846/Recommendations-for-Prevention-and-Control-of?autologincheck=redirected>